Improving First Call Resolution (FCR) Performance

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Business Case

Medicare call centers play a critical role in ensuring beneficiary access to plan information and maintaining CMS compliance standards. Currently, a low First Call Resolution (FCR) rate of 62% drives excessive repeat call volume, which significantly increases labor costs and creates a high Cost of Poor Quality (COPQ) through operational inefficiency.

Beyond direct costs, FCR is a primary driver of beneficiary experience, particularly during high-volume periods like Open Enrollment. Low resolution rates negatively impact CMS Star Ratings; since even a one-star decline can lead to a substantial reduction in federal quality bonus payments and member retention, improving FCR is a financial and strategic necessity. Increasing FCR to 75% will reduce cost-per-call, safeguard revenue tied to compliance and align performance with top tier industry benchmarks.

Problem & Goal Statement

- **Y Metric:** First Call Resolution (FCR) Rate (%)
- **Problem**: The current First Call Resolution (FCR) rate for FY2026 Q1 is 62%, which is below the desired performance level and results in repeat calls and operational inefficiency.
- **Goal**: Increase the First Call Resolution (FCR) rate from 62% to at least 75% by October 15, 2026 (Open Enrollment). The target of 75% aligns with industry benchmarks (70–79%) and represents a realistic and measurable improvement from the current 62% baseline.

Operational Definition

First Call Resolution (FCR) Rate is defined as the percentage of beneficiary calls that are fully resolved during the initial call.

As measured by:
Using the variable **FCR_Success** in the call management dataset.

A call is coded by CRM system as:
1 (Success) if the call is marked as FCR_Success = Yes in the dataset.
0 (Failure) if the call is marked as FCR_Success = No in the dataset.

FCR Rate is calculated as:

Total Calls with FCR_Success = Yes / Total Calls × 100

All calls included in FY2026 Q1 dataset (October–December)

Current State

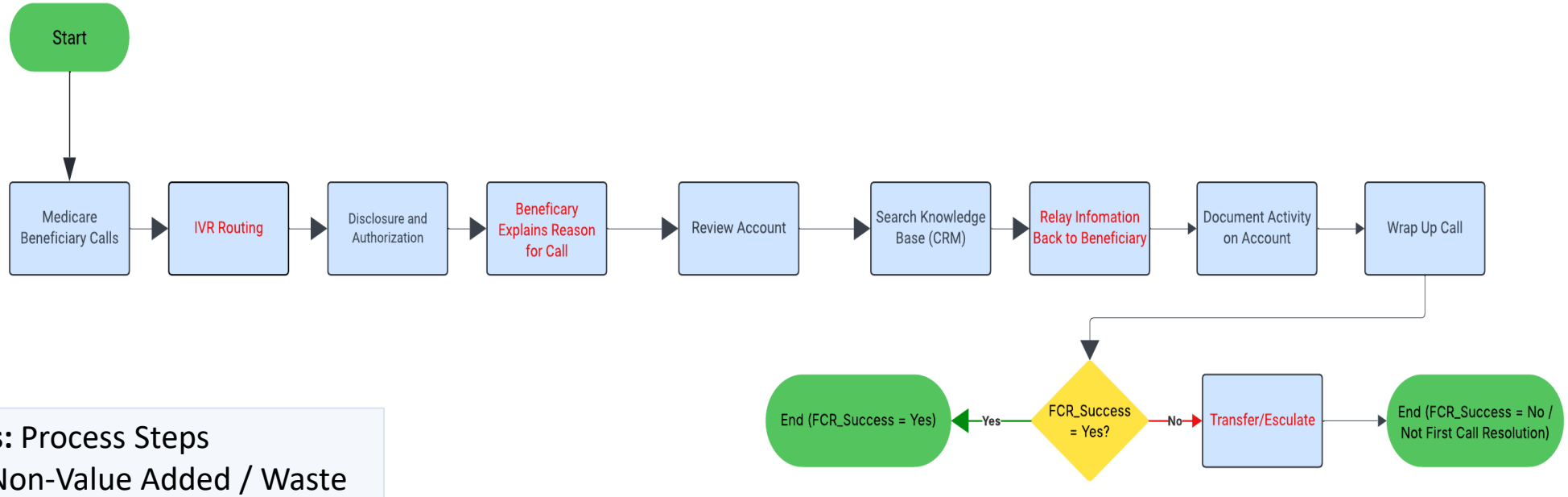
- **Sample Size (FY2026 Q1):** 300 Calls
- **Baseline FCR (FY2026 Q1):** 62%
- **Non-FCR (Defect) Rate:** 38% (Defect = any call where FCR_Success = No)
- **DPMO:** 380,000
- **Sigma Level:** 1.8
- **Goal Sigma Level :** 2.2

The current process is operating at a sigma level of 1.8, indicating high variability and a significant opportunity for improvement. A defect rate of 38% highlights that more than one third of calls are not resolved on first contact, driving repeat call volume and inefficiencies.

Critical to Quality (CTQs)

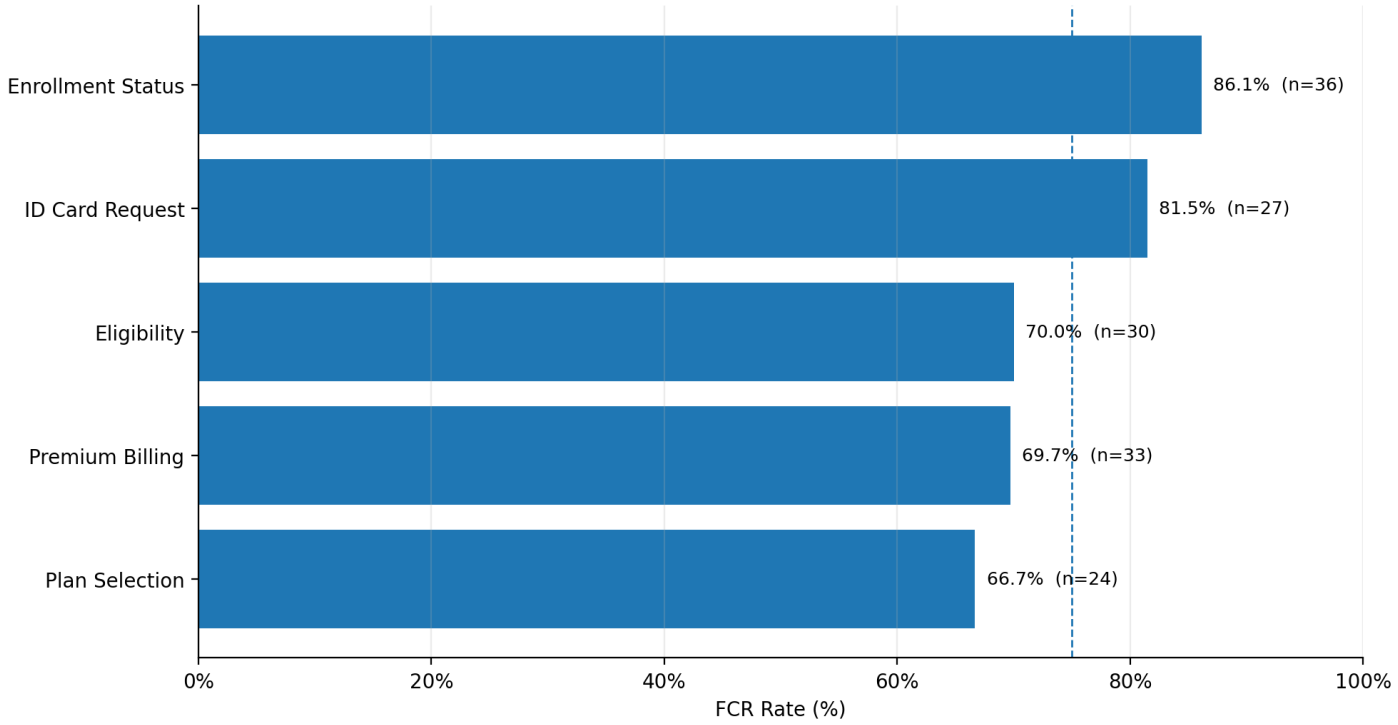
CTQ Category	Measurement Method	Measurable Specification (Target)
First Call Resolution (Primary Y)	CRM FCR_Success Variable	**≥ 75%** of calls resolved on first contact.
Average Handle Time (AHT)	Timestamp data from CRM	**≤ 13.0 minutes** (Aligned with the average for successful FCR calls).
Call Transfer Rate	Escalation/Transfer flag in dataset	**≤ 15%** (Reducing transfers directly supports higher FCR).
Information Accuracy	QA Audit Score	**≥ 95%** accuracy on Plan Selection and Premium Billing inquiries.

Current State – Process Map



- **Blue Boxes:** Process Steps
- **Red Text:** Non-Value Added / Waste (e.g., IVR Routing, Transfers).
- **Yellow Diamond:** Decision Points / Inspection.
- **Green Ovals:** Beginning/End of Interaction

Root Causes: X1- Plan Selection



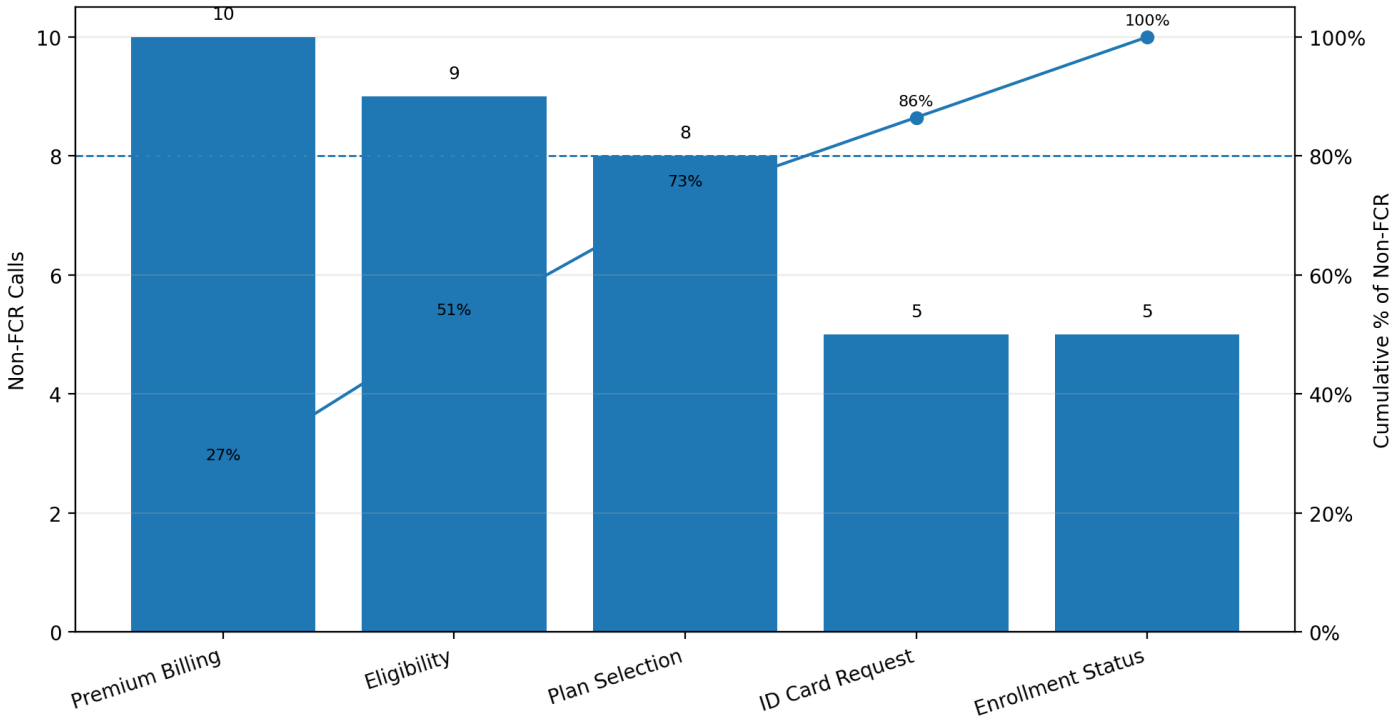
What the data shows

- Plan Selection had the lowest observed FCR at 66.7%, followed by Premium Billing at 69.7%.
- Enrollment Status performed best at 86.1%, showing resolution performance is not uniform across call types.
- The biggest improvement opportunity is to strengthen support for the three lowest-performing inquiry types.

Conclusion

Lower FCR in Plan Selection and Premium Billing suggests that higher complexity inquiries require additional decision support and agent knowledge. These categories represent the greatest opportunity for targeted process improvements.

Root Cause:X2- Premium Billing, Eligibility, Plan Selection



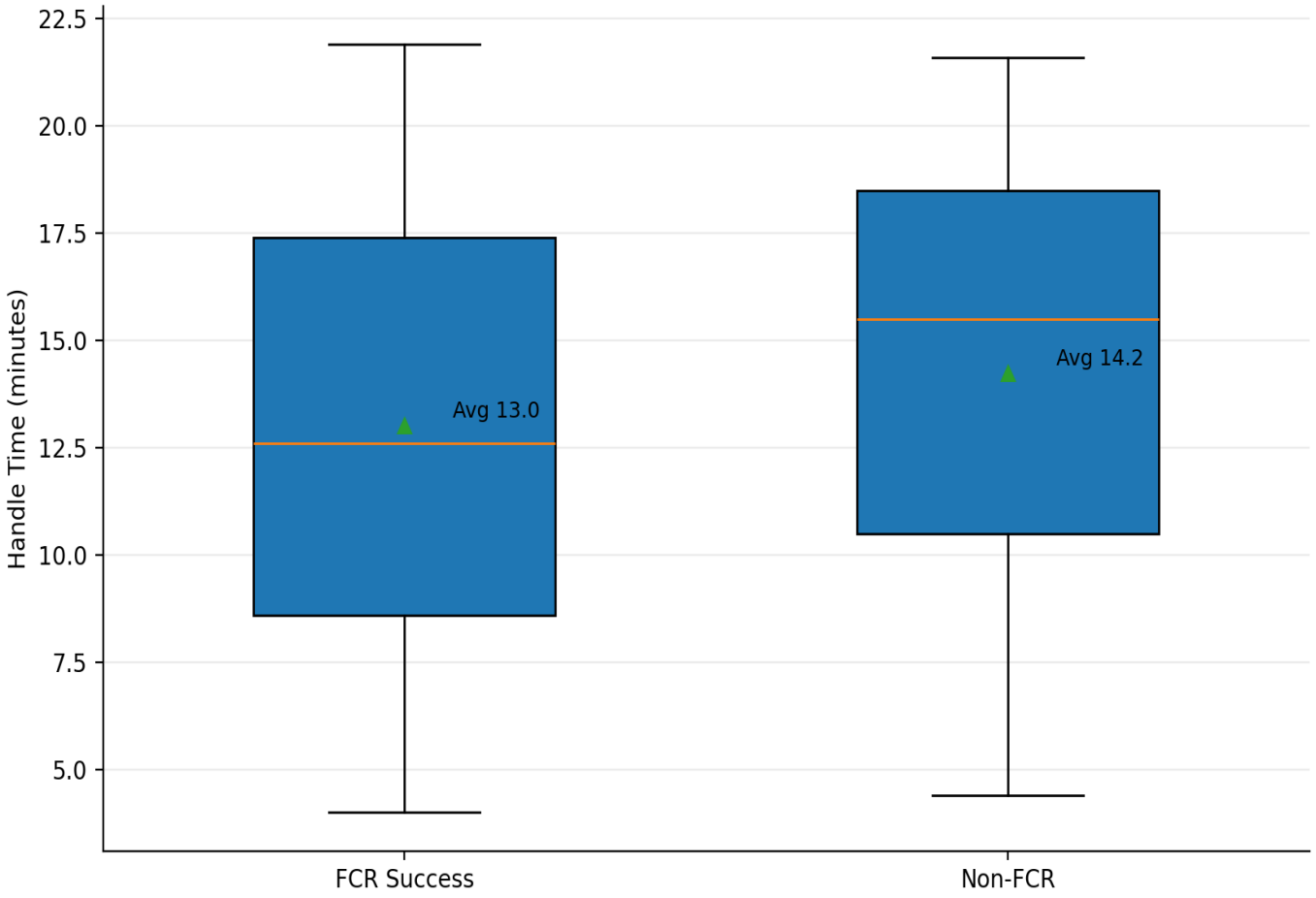
What the data shows

- Premium Billing, Eligibility, and Plan Selection account for 27 of the 37 non-FCR calls in the sample.
- Those three inquiry types represent 73% of all observed failures.
- A focused improvement plan should prioritize the largest failure buckets before broad changes.

Conclusion

The concentration of 73% of non FCR calls within three inquiry types indicates that performance issues are not evenly distributed. Targeting these high volume failure categories will yield the greatest impact on overall FCR improvement.

Root Cause: X3-Process Inefficiency/Handle Time



What the data shows

- Non-FCR calls averaged 14.2 minutes compared with 13.0 minutes for calls resolved on the first contact.
- The longer duration suggests more searching, rework, or back and forth before closure.
- Standard call guides and clearer knowledge support can shorten effort on complex calls without rushing the member.

Conclusion

Longer handle times for non FCR calls suggest inefficiencies such as knowledge gaps, system navigation challenges, or unclear processes. This indicates that additional time is not improving resolution but instead reflects avoidable rework.

Root Cause Analysis: The 5-Whys

Problem: Plan Selection, Premium Billing, and Eligibility account for **73% of FCR failures** (27/37 calls).

Why are these three categories failing to resolve on the first contact?
Answer: Front-line agents are unable to provide definitive answers for these complex inquiry types during the initial interaction.
Why can’t they provide a definitive resolution?
Answer: The current Process Map shows a generic "Search Knowledge Base" step that lacks specific, guided workflows for high-complexity billing and plan logic.
Why is searching the knowledge base ineffective for these topics?
Answer: Standardized call guides and "job aids" have not been developed for these specific high-volume failure buckets. Information with plans, premiums, and eligibility may change from year to year.
Why haven't targeted guides been developed?
Answer: Training and Quality Assurance (QA) historically focused on "Average Handle Time" (AHT) rather than identifying the root causes of repeat contacts.

Root Cause: Lack of category-specific "decision-support" tools and targeted coaching for high-complexity inquiries

Recommendations

Each recommendation below ties directly to a root cause observed in the analysis phase.

Recommendation	Why it matters	Timing / Cost
(X1): Create targeted scripts and job aids for Plan Selection, Premium Billing, and Eligibility inquiries..	These were the lowest-performing inquiry types and drove most non FCR volume.	2–3 weeks Low cost
(X2): Prioritize training and support for high-volume failure categories (Premium Billing, Eligibility, Plan Selection).	These categories account for the majority of non-FCR calls, so targeted improvements will have the highest impact on overall performance. Monthly results show a clear dip during the highest pressure period of the quarter, which is Open Enrollment (Oct-Dec).	3–4 weeks Medium cost
(X3): Add weekly QA review of long-handle non-FCR calls and incorporate findings into coaching.	Non-FCR calls had longer handle times, indicating rework, inefficient knowledge use, or unclear resolution pathways.	2 weeks to launch Low cost

Control Plan to Sustain Improvement

The goal of this plan is to sustain a **Sigma Level of 2.2** and an **FCR rate ≤ 75%**

Metric	Owner	Frequency	Reaction Plan (If Target is Missed)
FCR by Inquiry Type (X1)	Ops Manager	Weekly	If FCR for Plan Selection/Billing < 75%, trigger immediate script review with Training Lead.
Non-FCR Root Cause Log (X2)	QA Lead	Weekly	If a specific category drives > 30% of failures, deploy a "Rapid Review" coaching module.
Long Handle Call Audit (X3)	Supervisors	Daily	If AHT exceeds 15 mins for non-FCR calls, conduct side-by-side coaching to identify system navigation gaps.
Peak Readiness Review	Training Lead	Monthly	Update Job Aids and Scripts if CMS plan logic or premium costs change before Open Enrollment.

RECOMMENDATIONS

Appendix A: Data Collection Sample

Ten sample rows from the uploaded dataset used to create the analysis charts.

Member_ID	Inquiry_Date	Inquiry_Type	Escalation	Handle_Time_Minutes	Documentation_Completeness	Knowledge_Base_Usage	Agent_Experience_Months	FCR_Success
100000	2025-12-21	Premium Billing	Yes	17.3	Complete	Yes	7	Yes
100001	2025-10-12	Premium Billing	No	4.6	Complete	Yes	2	Yes
100002	2025-11-23	Plan Selection	Yes	14.6	Complete	Yes	11	Yes
100003	2025-11-05	Plan Selection	Yes	21.2	Complete	Yes	7	Yes
100004	2025-12-17	Plan Selection	No	4.8	Complete	Yes	25	No
100005	2025-12-20	Plan Selection	No	19.5	Complete	No	3	Yes
100006	2025-10-11	Plan Selection	No	19.6	Complete	Yes	24	No
100007	2025-10-27	Enrollment Status	No	16.6	Complete	Yes	11	Yes
100008	2025-10-21	Eligibility	No	8.9	Incomplete	Yes	15	Yes
100009	2025-10-30	ID Card Request	No	18.5	Complete	Yes	21	No